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Case Number: 24VECV02050 Hearing Date: September 8, 2026 Dept: O

SUPERIOR COURT OF THE STATE OF CALIFORNIA

COUNTY OF LOS ANGELES - NORTHWEST DISTRICT

SCOTT BADEN,

Plaintiff,

vs.

JEFFREY LAKE, M.D.; THE REGENTS OF THE UNIVERSITY OF CALIFORNIA; and DOES 1 through 100, Inclusive,

Defendants.

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CASE NO.: 24VECV02050

ORDER GRANTING DEFENDANT JEFFREY LAKE, M.D.'S MOTION FOR SUMMARY JUDGMENT

I. BACKGROUND

Scott Baden ("Plaintiff"), successor in interest to decedent Shaun Baden ("Decedent"), brings this action for medical malpractice on behalf of Decedent and himself against Jeffrey Lake, M.D. ("Dr. Lake") and The Regents of the University of California ("Regents") (jointly, "Defendants").

Dr. Lake moves for summary judgment.

II. PROCEDURAL HISTORY

On May 2, 2024, Decedent filed a Complaint for medical malpractice.

On June 10, 2024, Dr. Lake filed an Answer.

On June 11, 2024, Regents filed an Answer.

On February 11, 2025, Plaintiff filed a FAC alleging (1) Wrongful Death Due To Medical Malpractice and (2) Medical Malpractice.

On February 27, 2025, Dr. Lake filed an Answer.

On March 26, 2025, Regents filed an Answer.

On June 3, 2026, Dr. Luke filed the instant Motion.

As of September 8, 2026, no opposition has been filed.

III. LEGAL STANDARD

The function of a motion for summary judgment or summary adjudication is to allow a determination as to whether an opposing party cannot show evidentiary support for a pleading or claim and to enable an order of summary dismissal without the need for trial. (See *Aguilar v. Atlantic Richfield Co.* (2001) 25 Cal.4th 826, 843.) In analyzing such motions, courts must apply a three-step analysis: “(1) identify the issues framed by the pleadings; (2) determine whether the moving party has negated the opponent’s claims; and (3) determine whether the opposition has demonstrated the existence of a triable, material factual issue.” (*Hinesley v. Oakshade Town Center* (2005) 135 Cal.App.4th 289, 294.) Thus, summary judgment is granted when, after the Court’s consideration of the evidence set forth in the papers and all reasonable inferences accordingly, no triable issues of fact exist, and the moving party is entitled to judgment as a matter of law. (See Code Civ. Proc. § 437c(c);[1] *Villa v. McFarren* (1995) 35 Cal.App.4th 733, 741.)

“On a motion for summary judgment, the initial burden is always on the moving party to make a prima facie showing that there are no triable issues of material fact.” (*Scalf v. D.B. Log Homes, Inc.* (2005) 128 Cal.App.4th 1510, 1519.) As to each claim as framed by the complaint, a defendant moving for summary judgment must satisfy the initial burden of proof by presenting facts to negate or establish an essential element. (See *Scalf v. D.B. Log Homes, Inc.* (2005) 128 Cal.App.4th 1510, 1520.) Courts “liberally construe the evidence in support of the party opposing summary judgment and resolve doubts concerning the evidence in favor of that party.” (*Dore v. Arnold Worldwide, Inc.* (2006) 39 Cal.4th 384, 389.) A motion for summary judgment must be denied where the moving party’s evidence does not prove all material facts, even in the absence of any opposition. (See *Leyva v. Superior Court* (1985) 164 Cal.App.3d 462, 475; *Salesguevara v. Wyeth Labs., Inc.* (1990) 222 Cal.App.3d 379, 384.)

Once the moving party has met its burden, the burden shifts to the opposing party to show via specific facts that a triable issue of material fact exists to a cause of action or a defense thereto. (See Code Civ. Proc. § 437c(o)(2).)

IV. ANALYSIS

A. Moving Defendants’ Undisputed Material Facts

The FAC asserts causes of action for wrongful death based on medical malpractice and survival action against defendants Dr. Jeffrey Lake and The Regents of the University of California. (See Def.’s Sep. Stmt. of Undisputed Material Fact (“UMF”) No. 1.) Decedent first presented to Dr. Lake on May 12, 2021, for bloody bowel movements and perianal itching. Examination revealed a fissure in the posterior anal canal, inflammation and perianal inflammation. Dr. Lake ordered topical Nifedipine cream to be applied three times a day. (See *id.*, No. 2.) Dr. Lake next saw Decedent on May 14, 2021. Examination revealed perianal inflammation was improved. Dr. Lake gave Decedent a Botox injection to the internal anal sphincter. Decedent was to continue the Nifedipine and return in 3 weeks. (See *id.*, No., 3.) When Dr. Lake saw Decedent on June 15, 2021, the rectal bleeding had stopped with Botox injections and Nifedipine. The anal pain was much improved. Examination revealed that the fissure in the posterior anal canal was improved. Decedent was to continue applying nifedipine ointment and return in 3 weeks. (See *id.*, No. 4.)

Dr. Lake next saw Decedent on December 14, 2021. He had some slight pain and bleeding with firm stools and perianal skin irritation. Examination revealed perianal inflammation. Dr. Lake ordered Nystatin powder for the perianal skin twice a day. (See id., No. 5.) When Dr. Lake next saw Decedent on March 11, 2022, following his hip replacement, Decedent had constipation resulting in a return of anal pain and slight bleeding near his anus, not near the fissure. Examination revealed normal appearing perianal skin and a fissure in the posterior anal canal. Dr. Lake prescribed topical Nifedipine ointment, to be applied rectally three times a day for 30 days with three refills. Decedent was advised to be on a high fiber diet. (See id., No. 6.) On March 25, 2022, Dr. Lake saw Decedent, who reported pain on defecation and anterior perineal irritation. Examination revealed normal appearing perianal skin and a fissure in the posterior anal canal. Dr. Lake gave Decedent a Botox injection to the internal anal sphincter. Dr. Lake prescribed topical Nifedipine ointment rectally three times a day and instructed Decedent to return in three weeks. (See id., No. 7.)

Decedent did not return until October 26, 2023, when Dr. Lake saw Decedent and his father Scott Baden. Decedent had recently undergone a screening colonoscopy that revealed a right sided adenocarcinoma. A subsequent CT scan showed evidence of liver metastases. Decedent had seen oncologist Dr. Shaye who was starting him on multi-agent chemotherapy. He denied any blood in his stool or change in his bowel pattern, but had less of an appetite recently. (See id., No. 8.) Dr. Lake explained to Decedent and his father chemotherapy was the primary treatment for metastatic colon cancer and surgery would be indicated if the tumor burden in his liver responded to treatment. Otherwise, no surgery was indicated unless Decedent was symptomatic from his cancer and palliative treatment was needed. (See id., No. 8.) Dr. Lake last saw Decedent on December 26, 2023. He still had pain defecating and intermittent bleeding and completed four chemotherapy courses for metastatic colon cancer. Dr. Lake injected Botox to the internal anal sphincter. Decedent was instructed to follow up with Dr. Lake in four weeks but didn't return. (See id., No. 9.)

The chemotherapy stopped working at some point according to Dr. Fakhri at City of Hope. Dr. Fakhri referred him to oncologist Dr. Dayyani at UC Irvine. He underwent alternative chemotherapy at UCI. In early December 2024 Decedent developed a metastatic bleed to his brain and became paralyzed. Further treatment was no longer effective, and he was placed on hospice care on December 21 at Cedars Sinai. He died on December 28, 2024, at Cedars Sinai Medical Center. The death certificate lists the immediate cause of death as Stage 4 colon adenocarcinoma for years. (See id., No. 10.)

Dr. Lake complied in all respects with the applicable standard of care for a colorectal surgeon practicing in Southern California in 2021 to 2023, in his care and treatment of Decedent. (See id., No. 11.) To a reasonable degree of medical probability, no negligent act or omission attributable to Dr. Lake caused or contributed to Decedent's death. (See id., No. 12.)

B. Moving Defendants' Motions for Summary Judgment is Granted.

The FAC's two causes of actions for medical malpractice rest on allegations that Defendants breached their standard of care in diagnosing and treating Decedent, resulting in his death on December 28, 2024. (See FAC, ¶¶ 7-8, 11.)

The elements of medical negligence are: "(1) the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise; (2) a breach of that duty; (3) a proximate causal connection between the negligent conduct and the resulting injury; and (4) actual loss or damage resulting from the professional's negligence." (Simmons v. West Covina Med. Clinic (1989) 212 Cal.App.3d 696, 701-702.) (1) "Both the standard of care and defendants' breach must normally be established by expert testimony in a medical malpractice case." (Avivi v. Centro Medico Urgente Med. Ctr. (2008) 159 Cal.App.4th 463, 467.) (2) (3) (4)

Thus, in a medical malpractice case, "[w]hen a defendant moves for summary judgment and supports his motion with expert declarations that his conduct fell within the community standard of care, he is entitled to summary judgment unless the plaintiff comes forward with conflicting expert evidence." (Munro v. Regents of Univ. of Cal. (1989) 215 Cal.App.3d 977, 984-985.) (1) An expert declaration, if uncontradicted, is conclusive proof as to the prevailing standard of care and the propriety of the conduct of the health care provider. (See Starr v. Mooslin (1971) 14 Cal.App.3d 988, 999.) (2) (3)

Causation must be proven "within a reasonable medical probability based upon competent expert testimony." (Jones v. Ortho Pharmaceutical Corp. (1985) 163 Cal.App.3d 396, 402.) "Mere possibility alone is insufficient to establish a prima facie case."

(Id.) “There can be many possible ‘causes,’ indeed, an infinite number of circumstances which can produce an injury or disease.” (Id., at 403.) “A possible cause only becomes ‘probable’ when, in the absence of other reasonable causal explanations, it becomes more likely than not that the injury was a result of its action.” (Id.) “The issue of causation may be decided as a question of law only if, under undisputed facts, there is no room for a reasonable difference of opinion.” (Zaragoza v. Adam (2025) 109 Cal.App.5th 113, 120.)

Dr. Lake argues he is entitled to summary judgment on Plaintiff’s medical malpractice claims based on the undisputed facts he complied with the applicable standard of care and did not cause or contribute to Decedent’s death. (See UMF, Nos. 11-12.) Dr. Lake submits the expert declaration of Glenn Ault, M.D., a physician certified by the American Board of Surgery and the American Board of Colon and Rectal Surgery who is familiar with the standard of care required of colorectal surgeons such as Dr. Lake. (See id., Nos. 2-10.) Dr. Ault reviewed Decedent’s medical records, his death certificate, and other material, to opine: “Dr. Lake complied in all respects with the applicable standard of care for a colorectal surgeon practicing in Southern California in 2021 to 2023, in his care and treatment of [Decedent] in connection with the subject care and treatment.” (Id., No. 11.) Dr. Ault supports his conclusion by explaining due to Decedent’s symptoms concerning bleeding clearly caused by an anal fissure, age, medical history, and positive response to the prescribed treatment, the standard of care for treating an anal fissure did not require Dr. Lake to order a CT scan, colonoscopy, or labs. (See id.) Dr. Ault opines that based on Dr. Lake’s compliance with the applicable standard of care, Decedent “did not die as a result of any alleged failure by Dr. Lake to order a CT or labs. The standard of care did not require that for treatment of anal fissures...to a reasonable degree of medical probability, no negligent act or omission by Dr. Lake caused or contributed to [Decedent]’s death, or any damages alleged by plaintiff.” (Id.)

The Court finds Dr. Lake meets his burden in showing there are no triable issues as to Plaintiff’s two medical malpractice claims as Dr. Lake’s evidence negates the breach of duty and causation elements. There being no opposition, Plaintiff fails to meet his burden of showing that triable issues exist. Thus, Dr. Lake is entitled to summary judgment on Plaintiff’s claims.

V. CONCLUSION

Based on the foregoing, the Court, the Court GRANTS Dr. Lake’s Motion for Summary Judgment.

IT IS SO ORDERED.

DATED: September 8, 2026

Hon. Michael R. Amerian

Judge, Superior Court

[1] All statutory references are to California codes unless stated otherwise.